St. Jude Academic Health Center - Division of Hospital Medicine

CLINICAL DOCUMENTATION — NARRATIVE SURGICAL DISCHARGE SUMMARY V1

Patient: Thomas Hernandez | **MRN:** PAT-60011 | **DOB:** 1967-06-11 | **Gender:** Male | **Doc ID:** NARRATIVE_0011
Encounter Date: 2025-04-22 | **Discharge/Note Date:** 2025-04-25 | **Provider:** Dr. Marcus Vance, DO

PREOPERATIVE DIAGNOSIS & PROCEDURE

The patient underwent uncomplicated laparoscopic cholecystectomy for symptomatic cholelithiasis under general endotracheal anesthesia.

HISTORY OF PRESENT ILLNESS

The patient is an adult who presented electively for surgical intervention following conservative management failure. Pre-admission testing confirmed that preoperative bun was 14.3 mg/dL. Preoperative metabolic screening showed a baseline potassium of 4.12 mmol/L. The patient was cleared from a cardiopulmonary standpoint and taken to the operative suite.

POSTOPERATIVE HOSPITAL COURSE

The patient was extubated in the operating room and transferred to the post-anesthesia care unit in stable condition. Postoperative recovery was uneventful. For pain and infection prophylaxis, the patient was prescribed Amlodipine 5 mg PO every morning. On postoperative day one, postoperative white blood cell count remained stable at 7.1 10*3/uL. Surgical incisions were clean, intact, and without erythema or purulent drainage.

POST-DISCHARGE MEDICATIONS

Discharge medications: The patient will take Cephalexin 500 mg orally QID for 10 days, and continue baseline Amoxicillin 250 mg PO TID for 7 days. All prescriptions were transmitted electronically to the patient's pharmacy.

ACTIVITY & WOUND CARE PLAN

Keep dressing clean and dry. No heavy lifting greater than 10 pounds for two weeks. Surgical clinic follow-up scheduled in 10 days.